1. PURPOSE

To prevent and control healthcare-associated infections (HAIs) in the ICU by implementing strict infection control practices, ensuring patient safety, and compliance with NABH and IPC guidelines.

2. SCOPE

Applicable to all ICU healthcare personnel including intensivists, resident doctors, nurses, respiratory therapists, technicians, housekeeping staff, patients, and visitors.

3. RESPONSIBILITIES

3.1 Intensivist / ICU In-charge

- Ensure compliance with ICU infection control policies
- Monitor infection indicators and corrective actions

3.2 Infection Control Officer (ICO)

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- Supervise implementation of ICU IPC practices
- Investigate outbreaks and recommend actions

3.3 Infection Control Nurse (ICN)

- Daily surveillance of ICU infections
- Hand hygiene and bundle compliance audits
- Staff training and reporting

3.4 ICU Nurses & Staff

- Strict adherence to hand hygiene and aseptic techniques
- Prompt reporting of breaches or incidents

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4. STANDARD PRECAUTIONS (MANDATORY IN ICU)

4.1 Hand Hygiene

- Follow WHO 5 Moments of Hand Hygiene
- Alcohol-based hand rub preferred; soap and water if visibly soiled
- Hand hygiene before and after each patient contact and procedures

4.2 Personal Protective Equipment (PPE)

- Gloves for all patient contact
- Gown, mask, eye protection as per risk assessment
- Proper donning and doffing to prevent contamination

4.3 Respiratory Hygiene

- Mask for patients with respiratory symptoms
- Closed suction systems for ventilated patients

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5. TRANSMISSION-BASED PRECAUTIONS

5.1 Contact Precautions

- For MDROs, open wounds, diarrhea
- Dedicated equipment and gloves/gown mandatory

5.2 Droplet Precautions

- For influenza, meningitis
- Surgical masks within 1 meter of patient

5.3 Airborne Precautions

- For TB, COVID-19 (as applicable)
- N95 mask and isolation/negative pressure room

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6. DEVICE-ASSOCIATED INFECTION PREVENTION BUNDLES

6.1 Ventilator-Associated Pneumonia (VAP) Bundle

- Head-end elevation 30–45 degrees
- Daily sedation interruption and weaning assessment
- Oral care with chlorhexidine
- Subglottic suctioning where applicable

6.2 Catheter-Associated Urinary Tract Infection (CAUTI) Bundle

- Aseptic catheter insertion
- Daily catheter necessity review
- Closed drainage system

6.3 Central Line-Associated Bloodstream Infection (CLABSI) Bundle

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- Maximal sterile barrier during insertion
- Chlorhexidine skin preparation
- Daily line necessity assessment

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7. ENVIRONMENTAL CLEANING & DISINFECTION

- Hourly cleaning of high-touch surfaces
- Daily and terminal cleaning protocols
- Approved disinfectants with correct dilution

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8. BIOMEDICAL WASTE MANAGEMENT

- Strict segregation at point of generation
- Color-coded bins as per BMW Rules 2016
- Sharps disposed immediately in puncture-proof containers

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9. LINEN MANAGEMENT

- Soiled linen handled with gloves
- No shaking of linen
- Transport in covered bags

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10. EQUIPMENT & DEVICE DISINFECTION

- Bedside equipment cleaned between patients
- Ventilator circuits as per policy
- Maintain sterilization and disinfection records

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11. ISOLATION PRACTICES IN ICU

- Dedicated isolation beds/rooms
- Signage displayed clearly
- Restricted visitor entry

12. ANTIBIOTIC STEWARDSHIP IN ICU

- Culture before starting antibiotics
- Review antibiotics at 48–72 hours
- De-escalation as per sensitivity

13. SURVEILLANCE & REPORTING

- Monitor VAP, CAUTI, CLABSI rates
- Monthly data analysis and reporting to ICC

14. OCCUPATIONAL HEALTH & SAFETY

- Staff immunization as per policy
- Needle stick injury reporting and PEP
- Regular health check-ups

15. TRAINING & COMPETENCY

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- ICU-specific IPC training
- Periodic competency assessments
- Mock drills and audits

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16. AUDIT & QUALITY IMPROVEMENT

- Hand hygiene and bundle compliance audits
- Corrective and preventive action (CAPA)

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17. DOCUMENTATION & RECORDS

- ICU infection surveillance forms
- Audit checklists
- Training attendance and incident records

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18. REFERENCES

- NABH Standards for ICU
- WHO ICU Infection Prevention Guidelines
- MoHFW, Govt. of India

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